

2.1 Criteria for Accepting and Retaining Residents

.....Revised 09/06/2018 Reviewed 09/06/2018

Policy Purpose

Merrill Gardens evaluates the care and service needs of a resident and/or potential resident within the licensing guidelines of the community. In addition to evaluation, this procedure secures a reservation for a new resident, and defines administrative procedures for the admissions process. Admission considerations, as well as long-term residency, include lifestyle and healthcare needs that fall within the limits of the community's license.

This initial interview is often the introduction to Merrill Gardens communities. An efficient process, and courteous, supportive attention reflects positively upon the interviewer, team members, the community, and our company. Indeed, our passion for excellence and teamwork is embodied within this initial process.

AZ: (803.C.1.i) *Merrill Gardens will not accept or retain residents that fall outside the scope of the Community scope of services as defined in R9-10-807(C). The suitability of a resident will be determined by reviewing the medical examination and the Resident Evaluation.*

Definitions

Evaluation Interview

Interview with potential resident, family and/or resident representative to determine the level of independence and assisted living care the potential resident wants and needs while residing at a Merrill Gardens community.

Assisted Living Care

A service designed for seniors to accommodate the need for assistance with activities of daily living, emphasizing personal choice, privacy, and resident dignity.

Community

The facility which provides services and/or housing to residents.

Licensed Community

For this manual, the term Licensed Community refers to our communities in all states who are licensed to provide Assisted Living Care. Terms used by individual states for regulatory purposes are as follows:

- Alabama, Arizona, Florida, Georgia, Washington - ALF: Assisted Living Facility (Alabama SCALF: Specialty Care Assisted Living Facility)
- California - RCFE: Residential Care Facility for the Elderly

- Nevada - RFG: Residential Facility for Groups
- South Carolina - CRCF: Community Residential Care Facility

Manageable incontinence

Incontinence controlled by use of toileting and/or absorbent products.

Non-ambulatory

Refers to residents requiring mobility aides. Also refers to residents with cognitive impairments. See state laws for specific applications.

Primary diagnosis

Resident's major health issue

Resident

The person residing in the Merrill Gardens community.

Unmanageable incontinence

Resident is unwilling or unable to manage incontinence

WA: Qualified Assessor- *The person responsible for completing a preadmission assessment of a prospective resident:*

- (1) Has a master's degree in social services, human services, behavioral sciences or an allied field and two years social service experience working with adults who have functional or cognitive disabilities; or*
- (2) Has a bachelor's degree in social services, human services, behavioral sciences, or an allied field and three years social service experience working with adults who have functional or cognitive disabilities; or*
- (3) Has a valid Washington state license to practice nursing, in accordance with chapters 18.79 RCW and 246-840 WAC; or*
- (4) Is a physician with a valid state license to practice medicine; or*
- (5) Has three years of successful experience acquired prior to September 1, 2004, assessing prospective and current assisted living facility residents in a setting licensed by a state agency for the care of vulnerable adults, such as a nursing home, assisted living facility, or adult family home, or a setting having a contract with a recognized social service agency for the provision of care to vulnerable adults, such as supported living.*

Guidelines

Basic Services

Included in our month-to-month rent are:

- Apartment accommodations.
- Up to three nutritious meals per day, served in our dining room.
- Room service provided at no charge for residents who become ill for up to 5 consecutive days. After that, or if a resident prefers dining in his/her apartment, a nominal charge is applied.
- Weekly housekeeping, including washing and changing linens and towels.
- Scheduled transportation throughout the week, and assistance in arranging transportation if we cannot directly provide it.
- Assistance in scheduling physician appointments.

- Use of common areas.
- Organized activities that promote independence and the maintenance of physical, mental, social, spiritual, and emotional health.
- Routine maintenance of apartment.
- All utilities, excluding telephone.
- Emergency call system.

Assisted Living Services

These services are packaged in Levels of Service that are priced according to the resident's needs, and the demands of the local marketplace. Our Licensed Nurse or qualified assessor conducts an initial evaluation within five days of move in. All residents receiving assisted living services will be charged for the service level determined by their Evaluation and Service Plan. These plans will be reviewed monthly. The resident is re-assessed at every 6 months, with changes in condition, and as required by state regulations.

AL, GA: Evaluation must be done prior to or at the time of move in.

GA: The resident must be re-assessed at least annually and with changes in condition. Residents and their representatives or legal surrogates will be informed, in writing, at least 60 days prior to changes in established charges and services.

All residents will be provided assisted living services, treatments, medications, diets, activities and other prescribed procedures documented in their service plan. Physician, as well as health care orders, will be used for updates. The General Manager (administrator) and Merrill Gardens team members will provide watchful oversight to help ensure that the resident has sufficient staff time, based on their needs, so that they can be:

- *Kept comfortable and clean*
- *Treated with dignity, kindness, consideration and respect*
- *Protected from avoidable injury and infection*
- *Given prompt unhurried assistance if the resident requires help with eating*
- *Given assistance with daily hygiene, including baths and oral care*
- *Given assistance in transferring and assisted self-preservation when needed*
- *Have oversight as to resident appearance and condition of health, and provided oversight with their medication to the extent required by their ability and regulations*
- *Be allowed proxy services under stipulations set forth in the Proxy Caregiver policy*
- *Be provided memory care services as outlined in the Memory Care Disclosure and Garden House policies*
- *Be meaningfully engaged in activities of interest and choice.*

Resident Criteria

All or portions of Merrill Gardens communities may be licensed, depending upon the states in which they operate. By definition, whether licensed or not, the housing arrangement is chosen voluntarily by the residents, and/or responsible party. Acceptance to and residence in a Merrill Gardens community requires the resident to meet certain criteria as mandated under state regulations and company policy. If the resident does not have a Durable Power of Attorney for Health Care and Finances, or Guardian/Conservator, the family/responsible party is encouraged to pursue. Garden House residents must have a Durable Power of Attorney for Health Care and Finances, or Guardian/Conservator.

AZ: (803.C.1.i)

1. *Prior to admission, the resident will be evaluated using Merrill Gardens EHR.*
2. *The resident will have their physician complete the Medical Health Statement, certifying they meet the requirements of residency.*
3. *The Community will strive to ensure the provisions of R9-10-807 are met.*
4. *The General Manager will determine if the resident is appropriate for residency at the Community based on information in both records.*
5. *If the individual is not yet a resident of the Community, then admission will be denied if not appropriate.*
6. *If the individual is a current resident, then the Community staff will notify the resident or representative.*
7. *The Community will follow the Residency Agreement requirements for termination of residency and R9-10-807 H-J.*

Responsible Manager: General Manager

GA: *Residents must have needs that can be met by the community staff available and in consideration of needs of other residents, need for self-preservation of other residents and the construction of the building and equipment. Residents requiring assistance with evacuation from others, assisted self-preservation, must reside in apartments on the first and second floors.*

LICENSING REQUIREMENTS

Each licensed community receives a state license, which includes a license number and licensed capacity for the building. Where applicable, non-ambulatory bed capacity may also be designated on the license, along with special additional licenses and Waivers/Exceptions (i.e., *Contracted for Assisted Living in WA, Hospice Waiver in CA*). In some states, the license identifies the number of people the community may accept with cognitive impairments and/or mobility aides (i.e., canes, walkers, wheelchairs) as related to accommodations for all apartments, regardless of location in the building. The framed license is displayed near the front desk.

PRIMARY CARE PROVIDER

Each resident must have a primary care provider at the time of move in. If a resident moves in to the community prior to having a physician, the Resident Care Director will provide the resident and the resident's formal representative (if they have one) with several names and telephone numbers of physicians in the area. The resident must then choose a primary care provider within twenty-four hours. If the resident does not choose a Primary Care Provider within twenty-four hours, they will then be sent to the emergency room for any health-related issues until a physician is appointed.

AL: *If the community has a Medical Director (Alabama SCALF), the Medical Director will be contacted for health-related issues until a primary physician is designated.*

MINIMUM AGE

In most cases, residents may be no younger than age 62, however, regulatory requirements and the Fair Housing Act may result in slight differences for each state. Exceptions may only be granted by the Vice President of Operations, and in compliance with regulatory limits.

CA: *Minimum age is 60.*

FL, GA, SC: Minimum age per regulation is 18 years of age or older, although Merrill Gardens minimum age requirement of 62 will be used.

FINANCIAL VIABILITY

Residents and/or responsible party must have the financial resources to pay published basic rates, plus any additional services that may be required. Merrill Gardens reserves the right to verify financial resources.

GENERAL HEALTH

Merrill Gardens maintains communities, which may be licensed to provide assisted living services. The communities are not skilled nursing facilities. In order to qualify and remain in a Merrill Gardens community, the resident must:

- Be free of serious illness or health conditions specifically prohibited by state regulation, and/or community policy as described in this manual.

GA: *The resident must not have active tuberculosis, or require physical or chemical restraints, isolation or confinement for behavioral control.*

- Have some level of independence in the performance of daily living activities. Supportive assistance is acceptable.

GA: *The resident must be physically capable of actively participating in transferring from place to place. They must be able to participate in the social and leisure activities provided in the community.*

- Not require 24-hour skilled nursing care. State requirements will apply.

GA: *Cannot require continuous medical or nursing care and treatment.*

- Have a licensed physician or nurse practitioner complete the Medical Health Statement (or state form) and, where applicable, a Tuberculosis (TB) test or chest x-ray prior to move-in. Health exams must be done prior to move-in. Health exams are only valid if done within 60 days prior to move in, or as defined in state regulations.

AL: *Health exams must be done within 30 days prior to move-in.*

CA: *The LIC 602a Physician's Report is used in place of the Medical Health Statement.*

GA: *A licensed physician, physician's assistant or nurse practitioner must complete the Physician's Medical Evaluation for Assisted Living and a Tuberculosis (TB) test prior to move-in. Medical exams are only valid if done within 30 days prior to move in. If the applicant is being evaluated pursuant to an emergency placement made at the request of the Adult Protective Services Section, Department of Human Services or another licensed facility requesting placement because of activation of its emergency relocation plan, the complete physical examination may be deferred for up to 14 days after emergency admission if no record of an exam or clinical record is available at the time of admission.*

- Absence of a dementia diagnosis does not necessarily mean the resident is inappropriate for participation in the Garden House program. Especially in the early stages of dementia, the family should be encouraged to seek a full diagnostic work up for the potential resident.

Garden House Admission Criteria

In addition to meeting the usual Resident Criteria listed in the above sections, residents/prospective residents of the Garden House are subject to the criteria below:

- Diagnosis or behavior requiring secured unit.
- Independent or requires up to moderate assistance with ambulation and transfers.
- Independent or requires assistance with bathing, personal hygiene, eating, dressing, and/or toileting.
- Continent or manageable incontinence.
- Stabilized, uncomplicated medical conditions and medication regime.
- Behaviors are manageable by staff and do not present a danger to self or others.

Reasons for denial of admission to the Garden House include, but are not limited to:

- Failure to meet participation criteria
- Failure to meet financial requirements
- Legal issues regarding guardianship
- Inability of the community to meet the needs of an individual regarding physical or psychological needs.

APPROPRIATE DIAGNOSES FOR GARDEN HOUSE

- Dementia (Alzheimer's Dementia or Senile Dementia of the Alzheimer's type – SDAT)
- Multi-Infarct Dementia (MID)

DIAGNOSES REQUIRING REVIEW FOR APPROPRIATENESS

- Creutzfeldt-Jakob Disease
- Huntington's Disease
- Progressive Supranuclear Palsy (PSP)
- Pick's Disease
- Normal Pressure Hydrocephalus (NPH) (without history of head trauma)
- Parkinson's when combined with dementia
- Dementias caused by exposure to toxins (other than alcohol)
- Stroke
- Down's Syndrome when Dementia is present
- Unspecified Progressive Dementia diagnosis without mental illness
- Wernicke-Korsakoff Syndrome (Korsakoff or Alcoholic Dementia)
- Post Traumatic Brain Injury
- Anoxia
- All forms of mental illness
- Other types of dementia upon Vice President of Operations review

GARDEN HOUSE DISCHARGE CRITERIA

- Constant bedridden state (over 14 days); exception if receiving Hospice services.
- Skilled care needs due to complex or unstable medical conditions, or complex medication regime.
- Inability to maintain weight and nutritional health status (i.e., requires calorie counts, tracking input and output, etc.)
- Unmanageable incontinence.
- Unstable, unmanageable psychiatric condition that presents a danger to self or others.
- Inability to benefit from the environment of the Garden House.
- Family's unwillingness to abide by the care practices offered within the program or community.

STAGING DEMENTIA

There are several methods of staging dementia. Merrill Gardens will use the three-stage system as defined by the Alzheimer's Association. The Alzheimer's Association staging system includes Early, Middle, and Late stages. There is not a clear division between stages, just as the individual resident may display characteristics from several stages. The staging system is a way to determine the behaviors and characteristics commonly expected and the approaches that may be successful. Residents move through the stages at different rates.

Early Stage:

- Memory loss may be the first noticeable change. Many residents have been able to conceal their memory losses for quite some time. The first part of memory to be impaired is the recent memory. Residents may forget to turn off a burner after cooking. This does not necessarily mean they have dementia, but becomes cause for concern when the forgetfulness interferes with everyday life.
- Residents may lose interest in activities they had previously been involved with. They may be more reclusive, or more anxious and easily upset.
- Determining correct change or balancing a checkbook may become more difficult.
- Residents may have difficulty finding the correct word during conversation, or may misname objects.
- Residents may show signs of poor judgment, such as making inappropriate purchases.
- Routine tasks, such as grocery shopping may take much longer to do. It is not uncommon for a normally short task to last all day.
- Residents have difficulty staying on task.

Middle Stage:

- Characteristics of early stage dementia become worse and physical control problems emerge.
- There is a significant increase in memory problems.
- Residents may become lost easily in their own apartment.

- There is increased confusion regarding time. Residents may wake from an afternoon nap and think it is time for breakfast.
- Residents may have difficulty sleeping at night.
- Behaviors such as agitation, anger, anxiety or suspiciousness may be more noticeable.
- Residents will need some assistance with personal care and other activities of daily living.
- An increase in supervision is required.

Late Stage:

- The characteristics of early and middle stage dementia worsen, requiring an increased dependence on others for all needs. Residents at this stage may not be appropriate for assisted living.
- Walking and independent eating abilities are lost.
- Swallowing problems may emerge.
- Speech becomes meaningless and eventually is lost.
- The resident becomes totally incontinent of bowel and bladder.
- Eventually, the resident fails to recognize family or themselves.

Health Conditions Guidelines

Any time a resident or prospect has health conditions or service needs that fall into one of the B or C categories, immediate involvement is required by the General Manager and/or Vice President of Operations, as appropriate.

Generally Allowed--(A)	Requires Review--(B)	Not Allowed--(C)
Self-Managed or needs assistance * 1. Medication 2. Bathing and dressing 3. Personal hygiene 4. Incontinence of bladder or bowel (self-managed) 5. Morning and evening routines 6. Escorting or assistance with orientation in the Community. 7. Has early stages of dementia or related disorders. 8. Oxygen administration (self-managed only) 9. Managed Diabetes (self-managed)	1. Any condition requiring extensive staff hours (continuous daily attention from LN prohibited in SC) 2. Brittle Diabetes (prohibited in SC) (or using capillary blood glucose meter in GA) 3. Catheters (self-managed) (External catheter care in GA) 4. Conditions monitored and administered by reliable skilled medical professional for: • Colostomy or Ileostomy • Gastrostomy • Oxygen (Initial order prohibited in SC) • Healing Wounds (<i>prohibited in AL</i>) 5. Contractures	Needs ongoing skilled medical care 1. Dermal ulcers: Stage 3 and 4 require medical relocation to higher care level. See exhibit. 2. Active communicable diseases /serious infection 3. Bedridden or unable to self-transfer (state limitations apply) 4. Inability to get along with others in the Community 5. Conditions that pose serious health or safety threats to themselves, or to any other resident or staff (example: combativeness) 6. Condition requiring care by a skilled nursing facility or hospital (example: post-operative/trauma recovery)

<i>*Requires a license or the limited involvement of a home health care provider. Regional Director of Operations must approve involvement of Home Health Agency in unlicensed Communities.</i>	6. Dermal Ulcers (<i>prohibited in AL</i>) Stage 1 or 2 (Decubitus ulcers) may be cared for in our Community by a licensed nurse, with physician's permission, within state regulations. See exhibit. (Stage 2, 3, 4 or multiples prohibited in SC) 7. Incontinence of bowel or bladder managed with assistance and resident cooperation 8. Enema/suppository administration 9. Injections managed with assistance (insulin, B12 & monthly injections only) (Routine IM or SQ other than 1x daily insulin prohibited in SC) 10. Intermittent Positive Pressure Breathing Machine 11. Medications that must be administered by someone else. (<i>prohibited in AL outside of GH</i>) 12. Ongoing skin care (widespread skin disorder prohibited in SC) 13. Minor or colonized infection 14. Tracheostomies (<i>prohibited in AL, CA</i>) (<i>assist prohibited in SC</i>) 15. Colostomy/ileostomy care (Emptying and replacing a colostomy bag in GA) 16. Irrigations (<i>prohibited in AL</i>) 17. Postural supports 18. Primary need for care stems from a dementia diagnosis (consider GH placement) 19. GA: Application of clean dressing-beyond first aid 20. GA: Collection of clean catch urine sample 21. WA: Gastrostomy (SC assist prohibited)	7. Naso-gastric tubes. (Straw & syringe also prohibited in SC) 8. Nasopharyngeal or tracheal suctioning. 9. Gross incontinence of bladder or bowel (not self-managed or resident uncooperative) 10. Digital stool removal 11. IV therapy/TPN (exception for IV may be made in some Hospice situations) (prohibited in SC) 12. Ventilator care 13. Liquid Oxygen 14. Total dependence for daily activities 15. Restraints 16. Uncontrollable wandering (may be appropriate for Garden House residence) 17. Management of infected wounds, or wounds requiring sterile procedures. except for first aid within 48 hours of injury (GA: sterile wound care) 18. Catheter insertion or irrigation 19. Cutting toenails of Diabetics (outside provider only) 20. Primary need for care results from an ongoing behavior caused by a mental disorder (uncontrolled symptoms prohibited in SC) 21. SC: Meds that require frequent dose adjustment, regulation or monitoring i.e., sliding scale insulin. 22. SC: Dependency in all ADLS for > 14 consecutive days
--	--	--

AL: The following are not permitted in Alabama-Conditions monitored and administered by a reliable skilled medical professional for colostomy or ileostomy, oxygen, and healing wounds, medications that must be administered by someone else (except SCALF), Staph infection or communicable disease,

tracheostomies, irrigations, and restraints. Stage 1 or 2 dermal ulcers may be cared for by a licensed nurse, with physician's permission, and an exception from the licensing authority.

Application for Residency

When a potential resident wishes to move in to the community:

1. Complete the evaluation and service plan with the potential resident, family and/or guardian or responsible party, as appropriate. While a screening interview may be conducted by phone, the formal evaluation interview must be done in person to help ensure adequate evaluation of the potential resident. Families have a tendency to overstate their loved one's capabilities.

The Licensed Nurse or qualified assessor will conduct the initial evaluation. Note that some questions may be answered by the evaluator's own observations (i.e., if the applicant is standing and moving independently, it is not necessary to ask if a wheelchair is required).

AL: *In addition to the MMSE and PSMS, a Behavior Screening form, aphasia screening, and Geriatric Depression Screening must be completed for Garden House admission.*

GA: *The community must complete an evaluation of the resident that addresses the care needs taking into account the resident's family supports, the resident's functional capacity relative to the activities of daily living, physical care needs, medical information provided, cognitive and behavioral impairments if any, and personal preferences relative to care needs.*

2. If appropriate, review Assisted Living Services with the applicant, and provide him/or her with a copy of the service plan. Be specific in defining the services and fees.
3. Provide applicant with the pre move-in packet, as appropriate.

Application Approval

The Service Plan must be reviewed, approved and signed by the General Manager or designated team member. This approval does not guarantee acceptance until all appropriate forms are completed and submitted, most importantly the Medical Health Statement/History & Physical (or state form).

CA: *Physician's Report LIC 602A is used in place of the Medical Health Statement. This form must be completed and signed by a licensed physician or licensed nurse practitioner. Health exams should be done prior to move-in. Health exams are only valid for 60 days prior to move in, or as defined in state regulations. Provide applicant with a self-addressed stamped Merrill Gardens envelope for return of this information directly to the community. Stress that a Tuberculosis (TB) test may be required by state law. On the form, list the contact person and phone number at the Merrill Gardens community for questions.*

GA: *Physician's Medical Evaluation for Assisted Living is used in place of the Medical Health Statement. This form must be completed and signed by a licensed physician, physician assistant or licensed nurse practitioner. Health exams should be done prior to move-in. Medical exams are only valid for 30 days prior to move in.*

Vacancies

In a stabilized community, vacancies may occur monthly. When asked, indicate tactfully that vacancies occur generally due to needs beyond the services provided by our community, relocation (closer to family or specialized medical services not available locally), or the resident's passing. (Try to avoid use of the word, death).

Resident File Set Up

As the move-in date approaches, the General Manager or designee should photocopy all forms and set up Future Resident Folder. There must also be a tracking system for medical exam results. The Resident Care Director is responsible for setting up the resident health record. See Resident Records Management, regarding required resident forms and distribution of copies. Be sure to keep Account records separate from your "Resident Records" File. The Resident Account Record is for the accounting department and to be used as a receipt.

DISCHARGES AND IMMEDIATE TRANSFERS

GA: *The community does not admit or retain residents who need more care than the community is authorized or capable of providing. Residents may not be retained if they do not meet the above requirements for admission or the resident is not ambulatory and not capable of assisted self-preservation and /or the community does not have staff available to evacuate all of the current residents to a point of safety within established fire safety standards.*

When a resident no longer meets the requirements for admission, a move out notice will be given in accordance with the Resident Agreement, and the community will assist the resident in making arrangements to transfer to an appropriate facility. The administrator or designated representative will inquire as to the resident's and responsible party's preference regarding the facility to which the resident prefers to be transferred. A copy of the relevant resident records will be forwarded to the receiving facility within 24 hours of transfer if requested. If the resident is unable to make their own decisions and has not designated responsible party, the community will make a referral to the appropriate social service agency.

Residents will be transferred immediately or as soon as possible if they are a danger to themselves or others or require emergency medical care in compliance with GA rules. The General Manager or on-site manager of the community must initiate an immediate transfer to an appropriate setting if the resident develops a physical or mental condition requiring continuous medical care or nursing care. It is the responsibility of the General Manager or on-site manager to continually observe the residents to determine that residents continue to meet the criteria for residency in an assisted living community. Should a resident develop a physical or mental condition requiring continuous medical care or nursing care, or if a resident's continuing behavior or condition directly and substantially threatens the health, safety and welfare of the resident or any other resident, or if the resident's needs cannot be met by the community, the General Manager or on-site manager should initiate immediate transfer of the resident to a more appropriate living environment.

Where immediate transfer is required to be made, the General Manager or on-site manager shall make arrangements for transfer in accordance with the admission agreement and must transfer the resident to an appropriate setting where the resident's needs can be met. Prior to making such transfer, the General Manager or onsite manager will:

- *Inform the resident and representative or legal surrogate, if any, of the reason for the immediate transfer*
- *Inquire as to any preference of the resident and representative or legal surrogate, if any, regarding the appropriate setting to which the resident is to be transferred*
- *Inform the representative or legal surrogate, if any, of the resident's choice regarding such transfer*

- *Inform the resident and the representative or legal surrogate, if any, of the place to which the resident is to be discharged*
- *Provide a copy of the resident file to the receiving setting within 24 hours of transfer; and*
- *Document in the resident's file the following:*
 1. *The reason for the immediate transfer;*
 2. *The fact that the resident and the representative or legal surrogate, if anywhere, is informed pursuant to this paragraph; and,*
 3. *Appropriate location and contact information regarding the place to which the resident is to be transferred or discharged.*

SC: *When the provision of care/services in the facility, combined with other appropriately licensed services, in accordance with facility policy, e.g., hospice, home health, as may be ordered by a physician or other authorized healthcare provider, does not meet the needs of the resident, or if any resident becomes in need of continuous medical or nursing supervision, or if the facility does not have the capability to provide necessary care/services, the resident shall be transferred within 30 days to a location which shall meet those needs. The administrator shall coordinate this transfer with the resident, next-of-kin/responsible party, and sponsor.*

A. *Residents shall be transferred or discharged only as appropriate per the provisions of the Resident's Bill of Rights. In cases of medical emergencies, immediate transfer is permissible; however, the family member, and the sponsor, if any, shall be notified at the earliest practical hour, but not later than 24 hours following the transfer.*

B. *Prior to discharge, the resident, his/her appropriate family member, and the sponsor, if any, shall be consulted.*

C. *Residents shall be transferred or discharged to a location appropriate to the residents needs and abilities. Residents requiring care and/or supervision shall not be transferred/discharged to a location that is not licensed to provide that care.*

D. *Upon transfer/discharge of a resident, resident information shall be released in a manner that promotes continuity in the care that serves the best interest of the resident.*

E. *Upon transfer/discharge, the community shall strive to ensure that medications, as appropriate, personal possessions and funds are released to the resident and/or the receiving facility in a manner that helps ensure continuity of care/services and maximum convenience of the resident.*

BACKGROUND CHECKS

GA: *Either a check will be made on the National Sex Offender Registry website coordinated by the Federal Bureau of Investigation, or a fingerprint records check will be completed. Applicants who are registered sex offenders or have committed another violent crime will not be admitted.*



Move-In

Resident Care Manual

2.2 Supporting Resident Move-In...Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens follows a detailed process that is efficient and supportive for the move-in of a new resident. Our goal is to provide new residents with a positive, stress-free experience that is well planned and considerate of the resident's needs. This procedure includes completion of all administrative activities, team member notification of resident needs, public relations activities, and move-in assistance as appropriate. This supports our mission of providing outstanding services and a vibrant community living environment.

Guidelines

Prior to Moving In: Scheduling the Move-In

Upon acceptance of a resident, an anticipated move-in date is communicated by the Community Relations Director to the General Manager and/or designated team member.

The designated team member sets up the resident file in anticipation of the move-in. See Resident Records Management for a description of resident forms. Also see the checklist on the form, Move-In and Orientation.

Upon approval of move-in, the Community Relations Director informs the resident and family of acceptance and date when the selected apartment will be available. New residents must be reminded to have their physician return the Medical Health Statement (or state form) to us prior to move-in. **Move-in day should focus on getting settled. Make efforts to get paperwork completed ahead of time.**

ASSISTED LIVING RESIDENTS

In addition to the Medical Health Statement (or state form), Assisted Living residents also need written physician orders for all medications, including over the counter medications and PRN (as needed) medications. Guidelines include:

- Medications must be in their original containers, and not expired. New prescriptions are preferred.
- Provide resident with information about preferred pharmacies. Encourage resident to obtain new prescriptions from preferred pharmacies due to compatibility and consistency with the Community's procedures for packaging, storage, dispensing, and medication documentation and disposal.
- In the event a non-preferred pharmacy is used, medications must be packaged and labeled appropriately.

Community Relations Director Preparations include:

1. Arrange for seating in the dining room for the resident, and alert table mates in advance.
2. Set up a new resident Welcome Packet to include in a folder:
 - Personalized, handwritten note card from General Manager
 - Resident's Handbook
 - Cable TV channel guide
 - This week's food service menu
 - Activity calendar and newsletter
 - Information regarding telephone service (if not already connected)
 - List of department heads, the community's central phone number and address, and include change of address cards
 - Maps or Chamber of Commerce related information
 - Merrill Gardens giveaways: i.e., pens, key rings, refrigerator magnets, notepads
3. Order new door and mailbox signage
4. Inform the Resident Welcoming Committee of the move-in date
5. Inquire about the resident's desired beverage and small snacks for move-in day (cookies, fruit, etc.) These should be placed in the resident's apartment on move-in day
6. Arrange for welcome gift

24 Hours Prior to Move-In: Coordination of Activities:

COMMUNITY RELATIONS DIRECTOR

- Meet with the General Manager, all department heads, and the Resident Welcoming Committee.
- Place beverages in the resident's refrigerator, ensuring it is on and in proper working order.
- Inventory and sign for keys and attach them to the Welcome Packet. If keys need to be made, coordinate with the Maintenance Supervisor.
- Place Business Office Resident file, keys and Welcome Packet in the resident's apartment.

ACTIVE LIVING PROGRAM DIRECTOR

- Prepare short biography on new resident. Mount photograph and biography on Welcome board placed in a common area within the week of move-in with resident's or responsible party's permission.
- Route copy of biography to appropriate team member.

HOUSEKEEPING SUPERVISOR

- Personally check and approve the cleanliness of the vacant apartment.
- Place a small "Welcome to Merrill Gardens" tent card in the bathroom.

- Strive to ensure that the apartment is stocked with toilet paper and some soap, in the event someone needs to use the restroom while they are moving in.

GENERAL MANAGER

- Inspect the apartment at the end of the day to see that it is ready.
- Leave personalized note.

Move-In Day: Resident Introductions and Orientation

COMMUNITY RELATIONS DIRECTOR AND/OR GENERAL MANAGER

- In the apartment, next to the paperwork and Welcome Packet, place a snack, such as a plate of cookies or a bowl of fruit for the resident and their helpers.
- Greet the resident and family at the door, and show them to the apartment.
- Review file with resident for missing information. Clearly mark needed information to facilitate the rapid completion of all administrative tasks. The goal is to shift the focus on getting settled and not on administrative obligations.
- Escort the resident to each meal, taking care to introduce the resident to his or her new table mates.

BUSINESS OFFICE DIRECTOR

- Complete paperwork/data entry to establish the resident in the system for:
- Rent & ACH
- Census report
- Inclusion into the appropriate department files. See Resident Records Management
- Accounting, using the Resident Account form to be placed in the Business Office file.

OTHER INTRODUCTIONS

- Department Heads will drop by the apartment within five days of move-in to introduce themselves.
- The Resident Welcome Committee volunteer will stop by the apartment.
- Front Desk receptionist will check in with the resident in the evening to see if everything is all right, make them feel welcome and safe. Checking in with the resident should continue each evening for the next five days, for both independent and Assisted Living residents.

Post-Move-In: Move-In Follow Up

In the days following the move-in, complete the following:

- After all move-in paperwork is completed, the General Manager gives completed file to the Business Office Director.
- The Business Office Director must then verify that the rent roll, Emergency Information Book and mealtime census report, etc., have been updated. Then and ONLY then will the file be placed in the filing cabinet. Keeping the file on the desk serves as a reminder to complete the process of putting the new resident into our system.

- The Active Living Program Director will personally visit the new resident for the first week to go over the day's planned activities on the Activity Calendar, encouraging him or her to participate.
- If the resident is participating in Assisted Living, watch health conditions closely and document to determine adverse effects from the move. Being alert to the new resident's response to the move is the responsibility of all team members. This is part of the initial re-evaluation period for the service plan. Use appropriate forms to document observations, etc.
- General Manager and all team members should assist the new resident to adjust to the community, and observe for changes in health conditions.

2.3 Resident Records Management

Revised 09/06/2018 Reviewed 09/06/2018

Policy Purpose

Records management helps ensure a systematic, consistent and confidential approach for managing resident information. This process provides for easy retrieval of assisted living resident information, tracking of illnesses, medications, health care services and provides historical documentation of physical, emotional, mental and social abilities. Licensing guidelines require easy access of records by authorized representatives of departments, authorized regulatory agencies, and authorized representatives for assisted living residents. Records must be maintained for at least three years (or as required by state) after the resident leaves the community. Maintaining appropriate records for each assisted living resident provides for regulatory compliance.

AZ: Records must be retained for at least 6 years after date of move out.

WA: Records must be retained for 5 years after resident moves out.

Guidelines

Records Maintained for Residents

Documentation of care provided each of our assisted living residents is a key element of delivering efficient and accurate services and maintaining compliance with state regulations. The following defines the forms and/or information required, and the file maintenance process. State regulation may require different set up of resident documentation books.

EMERGENCY INFORMATION BOOK

Located in a secure area near the front desk, this binder contains emergency contact information that would be needed in case of evacuation or by paramedics for all residents residing in the community.

ASSISTED LIVING RESIDENT CHART

Located in a secure area such as the Assisted Living office, each resident residing in a unit licensed for assisted living will have an individual chart. The chart should be a three-ring binder with section dividers. Assisted Living resident health information is filed in each resident's chart. Components of the chart may be in separate locations, such as a binder for current Service Plans and Service Logs. Completed paper Service Logs, Medication & Treatment sheets are to be filed in the resident chart. An overflow file may be kept on the premises in a locked area. Upon discharge, the chart information is kept on file at the community for at least 3 years, or as required by state regulations.

Resident Information Documentation

Forms *Use state provided forms as appropriate	Business Office Resident File	Emergency Book (Front Desk)	AL Chart (AL office)	EHR
Evaluation			✓	✓
Service Plan (required for all residents in fully licensed Communities)			✓	✓
Resident Account	✓			
Emergency Contact	✓	✓	✓	✓
Advance Directives/POLST copy	✓	✓	✓	
DPOA/Guardian/Conservator copy	✓		✓	
Insurance card copy	✓	✓	✓	
Medical Health Statement*		✓	✓	
Release of Resident Information and Confidentiality	✓		✓	
Residency Agreement	✓			
Move In & Orientation Checklist	✓			
Activity Interests			✓	
Resident's Bill of Rights*	✓			
Resident Service Log			✓	
Resident Progress Notes			✓	
Incident Report*				✓
Release or Destruction of Medication			✓	
Negotiated Service Agreement			✓	
Care Alert			✓	
Mini Mental Status Exam (MMSE)			✓	
<i>GA: Admission & Discharge Log</i>	✓			✓
<i>GA: Proxy Caregiver Consent</i>			✓	
<i>CA, GA: Inventory of Valuable Personal Items</i>	✓			
<i>GA: Personal Needs Allowance Waiver</i>	✓			
<i>GA: National Sex Offender Registry findings or fingerprint check</i>	✓			
Physician Orders & Changes			✓	
Pharmacy Records for Prescriptions			✓	

Assisted Living Chart Order

Place state specific forms in appropriate sections as listed below:

ADMISSION RECORD

- Photo ID
- Emergency Contact
- Healthcare POA /Guardian/Conservator copies
- Advance Directives/POLST/DNR Acknowledgement (Resident Agreement Exhibit)
- Copies of any Advance Directives/POLST/or DNR documents
- Insurance card copies
- Confidentiality & Disclosure

- Apartment Entry Authorization (Resident Agreement Exhibit)
- **CA:** Consent for ER Medical Treatment (LIC627c)
- **CA:** Release of Resident Medical Information (LIC 605a)
- **CA:** Personal Rights (LIC613c or RA exhibit)
- **CA:** Resident Personal Property & Valuable Inventory (LIC 621)
- **CA:** Telecommunications Device Notification (LIC 9158)
- **GA:** Proxy Caregiver Consent

HISTORY/PHYSICAL

- Medical Health Statement/ H&P (or state required form)
- **CA:** Physician's Report for RCFE (LIC602A)
- **GA:** Physician's Medical Evaluation
- Discharge/transfer records from other settings

PHYSICIAN'S ORDERS

- Health Care provider orders
- Faxed Notification to healthcare provider (including Negotiated Service Agreement)
- Dietary Communication and Diet Orders
- **AZ:** Vaccine Acknowledgement form
- **CA:** Toxic Chemical Storage orders
- **CA:** PRN Authorization letter
- **NV:** MD Notification of Missed/Refused Medication

PROGRESS NOTES

- Assisted Living: Monthly Care Review
- Progress Notes-current notes are in Service Log binder
- Completed Service Logs
- Care Alerts
- **AZ:** Pre & Post Transfer Evaluation forms
- **CA:** Unusual Incident/Injury Report (LIC 624)

VITAL SIGNS

- Weight record
- **AL:** Vaccination Records
- **NV:** NOT USED

MEDICATION/TREATMENTS

- Completed medication & treatment records-current records are in Med book
- Release/ Destruction of Medication
- **CA:** Centrally Stored Medication Log (LIC622)-current log in med book

- **NV:** *Ultimate User Medication Authorization*

LAB & SPECIAL REPORTS

- Copies of lab results, tests and radiology reports

CONSULTS/MISCELLANEOUS

- Copies of Assessments, Service Plans and progress notes for resident arranged services such as Home Health, Therapy, Mental Health, Podiatry, Dentistry, etc.

EVALUATION/CARE PLANS

- Resident Evaluation
- Service Plan-current plan is in Service Log binder in sheet protector
- Activity Interests
- Food Preferences
- Negotiated Service Agreement
- Mini Mental Status Exam (MMSE)
- **CA:** *Pre-Placement Appraisal (LIC 603)*
- **GA:** *Proxy Caregiver Plan of Care*
- **WA:** *Family Assistance with Medications or Treatments plans*

Documentation in the assisted living resident chart is known as “documentation by exception”. That means that information is documented that is unusual rather than routine.

This includes, but is not limited to:

- Incidents – description of incident and notation of reaction/result of incident including notification of primary physician & family, and action taken.
- Care issues – resident refusing care, care that exceeds our typical level of services or a change in services
- Change in condition - resident is running a temp, complaining of pain or has indications of flu, etc.; and
- Medical emergencies

Progress Notes

Assisted Living (AL) team members will document observations on the Progress Note provided for all residents on the AL program or residing in a licensed unit.

- The goal of the AL team is to document facts, observations, unusual occurrences and communication about the assisted living resident on the Progress Notes form. Action taken is noted along with resident response.
- Refusals of service are to be documented on the Progress Notes form along with the plan of action taken to alleviate the situation.
- Entries are to be made in black ink; dated with the month, day & year, and time of day if possible. Entries must be initialed with the identifying signature on the same page.
- Items noted that need further action, need to be communicated to a management team member.

- The Resident Care Director, Garden House Director, or a management team member will take appropriate action to rectify a situation that has been documented on the Progress Notes. They will also communicate with the responsible person and/or physician. Any subsequent actions will be documented by a management team member on the Progress Notes.
- When the Progress Notes form is complete, it is filed in the resident's chart and a new form is started.

Correcting Documentation Errors

Documentation errors are to be corrected by the person making the error.

A single line is to be drawn through the incorrect documentation. The date and initials of the person making the correction should be written near the lined-out documentation.

Records Updating

Records should be periodically reviewed and thinned from current active files as listed below. The General Manager will periodically review files to help ensure that they are current. Remove duplicate copies before storing information in an inactive file. Update files as follows:

- The resident Health Chart may be thinned every six months and stored in the Assisted Living historical file.
- Review the Emergency Information Binder monthly to update.

Record Storage for Current Residents

The original Physician Health Statement or H&P, Evaluations, and Service Plan should be retained in the active chart in plastic sheet protectors during the resident's tenancy. Thinned records are to be retained in the community in a historical file for each resident.

Record Storage after Move Out

Upon departure of a resident, remove all resident records from active files and historical files and place in a discharged resident file. Store discharged files by the year of discharge. All resident records must be maintained for at least three years (or as required by state) after the resident no longer resides at the Community then may be destroyed.

AZ: *Records must be retained for 6 years after resident moves out.*

SC: *Upon discharge, the resident record should be completed within 30 days and filed in a closed file maintained by the licensee. Resident records will be maintained for at least 6 years after the resident moves out. Other regulation-required documents, such as fire drills, activity schedules etc. must be retained at least 12 months or since the last DHEC general inspection, whichever is longer.*

WA: *Records must be retained for 5 years after resident moves out.*

Confidentiality

The Community shall maintain confidential records providing access to files on an as needed basis or as mandated by state law. Access is limited to:

- Appropriate team members
- Resident
- Resident's representative/responsible party (legal guardian, conservator, spouse, or Power of Attorney)-check legal documents for authority.

- State or federal regulatory agencies
- Ombudsman (with written permission of the resident/responsible party)
- Family members or friends who have written permission from resident or legal representative.

Electronic Health Records

Electronic Health Records (EHR) will be used to document community evaluations and service plans. The Resident Care Director and Garden House Director (where staffed) will be responsible for maintaining the EHR for each assisted living resident. A paper copy of the current service plan will be maintained in each assisted living resident's chart. Electronic health records will be retained for at least three years (or as required by state) after the resident no longer resides in the community, then may be deleted.



Move in and Orientation Checklist

Click or tap here to enter text.

Resident Name(s): First

Middle

Last

Date of Move-in: Click or tap here to enter text.

Apartment Number: Click or tap here to enter text.

File in front of Business office folder

Pre-Move-In

Complete paper work: Signatures, file(s) set up to include: (Use state provided forms as appropriate)

- ☐ Resident Account
- ☐ Resident Evaluation & Service Plan
- ☐ Emergency Contact
- ☐ Medical Health Statement
- ☐ Release of Resident Information and Confidentiality
- ☐ Residency Agreement
- ☐ Resident Bill of Rights
- ☐ Resident Rights Agencies
- ☐ Resident Handbook

State Specific Forms:

- ☐
- ☐
- ☐
- ☐
- ☐
- ☐

Other:

- ☐ DNR Order, DPOA, Living Will, etc.
- ☐ Activity Interests (no signature needed)
- ☐ Alert Department Heads & Welcome Committee
- ☐ Update Resident Roster
- ☐ Door Signage
- ☐ Name on mailbox
- ☐ Dining seat assignment, if applicable
- ☐ Housekeeping Welcome Card in bathroom with toilet paper and soap
- ☐ Order Medications for med management
- ☐ Set up Health Chart

Post Move-In

- ☐ Review Health Evaluation & Service Plan
- ☐ Print Service Plan and Task Log
- ☐ Triple Check meds/orders/MAR
- ☐ Complete any outstanding paperwork
- ☐ Housekeeping schedule and laundry facilities
- ☐ Activities Interest Form
- ☐ Photo taken for Welcome Board/Photo Album

- ☐
- ☐

Orientation to Apartment:

- ☐ Appliances, light switches
- ☐ Heating and/or air conditioning controls
- ☐ Emergency Response System/ Pull Cord
- ☐ Fire exits and evacuation procedures
- ☐ Pet Policy, if applicable
- ☐ Orientation and tour of community
- ☐ Welcome Packet and gift

- ☐
- ☐
- ☐

Miscellaneous:

- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐
- ☐



2.4 Decision Making.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens supports resident independence in decision making. We also recognize that there may be times when the resident is unable or unwilling to act as their own responsible party. We will respect the resident's wishes in this area as communicated to our team members whenever possible within the law and Merrill Garden's policy. Nothing in this policy is to be interpreted as legal advice. When there is a question of who is able to make decisions on the resident's behalf, the advice of an attorney may be appropriate. Since applicable regulations may differ from state to state, this policy is intended to be a general guideline. If there is a conflict between this policy & local regulations, the local regulations are to be followed.

Definitions

Hierarchy

The order in which each person is contacted for decision making. If the first person is unable or unwilling, the next on the list must be contacted until a surrogate decision maker is identified.

Incapacitated

Unable to understand choices, make a decision and communicate the choice. Incapacity may be defined in the Power of Attorney. Generally requires the determination to be made by the resident's primary physician (or by two physicians).

Incompetent

Incapacity determined by the court.

Responsible party

The person or persons designated by the resident to make decisions on their behalf. The resident may be their own responsible party.

Surrogate

A person or persons other than the resident who is/are designated in writing (or via the hierarchy in absence of a designee) to make decisions for the resident if the resident is unable or unwilling to make a decision.

Guidelines

Resident as Responsible Party

The resident is the primary decision maker and responsible party unless they are unwilling, unable or designate another responsible party in writing. The resident is to be included in any decision making whenever practical. A resident is presumed to have the capacity to make decisions if they are able to

understand the nature and consequences of different options, can make a choice among those options, and can communicate their choice.

Power of Attorney

If a resident has an executed Power of Attorney, a copy of the document must be provided to the community. A copy will be kept in the health chart. A Power of Attorney must have a health care clause or be a Health Care Power of Attorney in order to allow the designee to make health care decisions for the resident as indicated in the document. A General Power of Attorney usually does not allow the designee to make health care decisions unless there is a specific clause to that effect. The resident has the right to override the decision of the person designated under the Power of Attorney at any time if they are competent.

Guardianship

If a resident has a Guardian (Conservator in some states), a copy of the guardianship document must be provided to the community. A copy will be kept in the health chart. A guardian is an adjudicated decision maker for the resident. A guardian may be a relative or other person appointed by a court. A guardianship is established when the resident is deemed by the court to be unable to make decisions for themselves. The guardianship may be limited to certain types of decisions, such as financial. The guardianship document will specify which types of decisions the resident is unable to make.

When Resident is Unable or Unwilling

A resident who is unable to make and communicate medical decisions is considered to be incapacitated. If the resident has provided Advance Directives in writing, they will serve as the basis for surrogate decision making. A surrogate decision maker must make decisions on behalf of the resident based on what the resident would have wanted had they been able to make the choice. All decisions are to be made in the best interest of the resident; meaning they should be what a reasonable adult in the same circumstances would decide. A resident who has the capacity to make a decision, but is unwilling to make the decision, may designate a surrogate decision maker.

Decision Making Hierarchy

If there is no documented surrogate decision maker, the right to make decisions for the resident falls to the next person in the hierarchy. Some states may have specific provisions that differ. Refer to state regulations. Generally, the decision making hierarchy is as follows:

1. Resident
2. Spouse
3. All Adult children
4. All siblings
5. Other family members
6. Friend

If there is disagreement among surrogate decision makers, or their decision does not coincide with the advance directive or what appears to be in the best interest of the resident, the physician should be contacted to discuss the decision with them.

If there are no surrogate decision makers, Adult Protective Services should be contacted & a temporary guardian may be appointed. In an emergency situation, the community will defer to emergency personnel for decision making.



Emergency Contacts

RESIDENT INFORMATION

NAME: _____ MARITAL STATUS: _____ ☐ MALE ☐ FEMALE
MOVE IN DATE: ____/____/____ APT.#: _____ PHONE: (____) ____-____
PRIOR ADDRESS: _____
SOCIAL SECURITY #: _____ MEDICARE #: _____
OTHER INS.: _____ OTHER INS. #: _____
BIRTH DATE: ____/____/____ ☐ VETERAN ☐ ADVANCE DIRECTIVES DISCUSSED
RESIDENT HAS: ☐ FINANCIAL POA ☐ HEALTHCARE POA/DPOA/GUARDIAN ☐ LIVING WILL ☐ DNR FORM
ALLERGIES: _____

IN CASE OF AN EMERGENCY, WHO SHOULD BE CONTACTED?

PREFERENCES	NAME / ADDRESS	PHONE / FAX NUMBERS INCLUDE AREA CODE
PRIMARY PHYSICIAN ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
ADDITIONAL PHYSICIANS MAY BE LISTED ON BACK OF FORM		
1 ST CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE: () - EMAIL:
2 ND CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE: () - EMAIL:
3 RD CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE: () - EMAIL:
ADDITIONAL CONTACTS MAY BE LISTED ON BACK OF FORM		
RELIGIOUS PREFERENCE: CLERGY/RELIGIOUS ADVISOR: ADDRESS: PHONE: DAY() - EVE () - FUNERAL HOME:	PREFERENCES PHARMACY: HOSPITAL: NURSING HOME: DEMENTIA UNIT:	

In case of emergency, I hereby authorize Merrill Gardens to summon help from the nearest or most readily available emergency or paramedic squad. I authorize following the squad's advice, including transfer to the nearest hospital, if recommended. I understand that I am responsible for costs incurred for such services. As the resident or authorized representative, I hereby give consent to provide all emergency medical or dental care prescribed by a duly licensed physician (M.D.) Osteopath (D.O.) or Dentist (D.D.S.) for the above named resident. This care may be given under whatever conditions are necessary to preserve the life, limb or well being or the individual named above.

SIGNATURE OF RESIDENT / RESPONSIBLE PARTY _____

DATE _____

OTHER HEALTH CARE PROVIDERS		
NAME- [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -
NAME - [practice] ADDRESS CITY STATE ZIP		DAY: () - EVE: () - FAX: () -

OTHER CONTACTS		
CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE:() - EMAIL:
CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE:() - EMAIL:
CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE:() - EMAIL:
CONTACT RELATIONSHIP ADDRESS CITY STATE ZIP		HOME: () - WORK: () - MOBILE:() - EMAIL:

2.5 Advance Directives-DNR/POLST

Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Residents have the choice of requesting a Do Not Resuscitate order in case of a medical emergency. Merrill Gardens respects this personal choice and maintains this information in strict confidence. However, in a medical emergency the community is required by law to perform basic first aid and treatment until paramedics arrive. The decision to withhold CPR or other resuscitation will be made by trained paramedics or other qualified medical professionals. Some states permit additional team members to withhold CPR based on appropriate documents being executed in advance.

Definitions

Advance Directive

General term for a document signed by the resident and notarized that outlines the resident's preferences regarding resuscitative measures and life sustaining treatment.

CPR

Cardiopulmonary Resuscitation. Chest compressions and rescue breathing performed when there is no apparent heartbeat and the resident is not breathing.

DNR

Do Not Resuscitate. Advance directive to not initiate CPR.

POLST

Physician's Order for Life Sustaining Treatment. A form expressing the resident's wishes regarding resuscitative measures and life sustaining treatment. It must be signed by the resident's medical practitioner.

Guidelines

Honoring a DNR (Do Not Resuscitate) Order

Residents who do not want CPR performed in the event of a medical emergency will have copies of their DNR order in the Emergency Information book and in their health chart.

WA: The POLST form is commonly used to summarize the resident's wishes regarding life-sustaining treatment. It is intended for use only by Emergency Medical Personnel.

The community will strive to ensure that the DNR order is:

- Signed by the resident or legally responsible party.

- Confidential: This is a highly confidential document, and must be treated with the utmost respect and privacy.
- **GA/WA:** Signed by the physician.

Merrill Gardens' Role

We will assist the resident in the honoring of a DNR in the following ways. Efforts will be documented on an Incident Report.

Below is the process we must follow by law. We will not deviate from this process for any reason, at anyone's request, including the resident's, his/her family, his/her physician (unless the physician is present at the time of the emergency), or his/her legal representative. 911 will be called unless the resident is on Hospice and the plan indicates that Hospice is to be called instead.

- If trained in CPR, a currently certified team member must perform CPR until paramedics arrive; where it is not obvious from physical observation of the resident's body (e.g. body is stiff, cool to the touch, blue or grayish in color, etc.) that such efforts would be futile and there is not a physician, registered nurse or physician's assistant on site to assess and provide other direction.

***FL:** The community shall have a functioning automated external defibrillator (AED) on the premises. The community is encouraged to register the location of the AED with a local emergency medical service. Staff may withhold or withdraw CPR or the use of an AED if presented with a valid order not to resuscitate.*

***GA:** CPR must be initiated unless the resident has a valid DNR order readily available. A caregiver may honor the DNR in good faith.*

***WA:** Caregivers may honor Section A of the POLST that refers to CPR-resuscitation. The community will perform CPR until paramedics arrive unless a valid POLST form is available that indicates "No CPR". If CPR has been started on a person with a valid POLST order, check to see if the resident's heartbeat has been restored. If there is a pulse, continue providing emergency care. If there is no pulse, stop CPR.*

- Basic First Aid will not be withheld. Efforts to stop bleeding or stabilize other injuries will be undertaken in accordance with American Red Cross standards. We will call 911 even if a resident with a DNR on file is found in an apparent medical emergency situation. CPR will be initiated for witnessed choking episodes or other accidents unless the POLST or Advance Directive indicates DNR-"No exceptions" in states where we can honor a DNR order **(WA/GA)**.
- Strive to ensure the format of the resident's DNR order form is acceptable to the local Fire Department Paramedical Unit. This is particularly necessary if they bring a form from outside the area.
***WA:** POLST forms are acceptable statewide.*
- Keep a copy of the resident's DNR order in the Business Office Resident file, the Emergency Information book and the health chart.
- Team members will have ready access to phone numbers for emergency medical personnel and the resident's file or appropriate emergency medical and contact information for each resident.

- Supply the resident, his or her family, and his or her physician with a copy of the DNR. The resident's copy should be placed in an envelope and attached to the back of his or her apartment bathroom door or other standard location. Paramedics may refer to it in this standard location. A copy will also be provided to the paramedics. The resident should retain the original form.
- Present a copy of the DNR order to the Paramedics upon their arrival.
- The Paramedics will make the decision to withhold CPR or other resuscitation efforts.
- Follow the appropriate procedure for a medical emergency or death, as appropriate, including notification of the General Manager and the family. Again, if the resident dies, **DO NOT TELL THE FAMILY ON THE TELEPHONE**. They must only be made aware of the emergency. If the family is away from the area, and the resident has been pronounced dead by a skilled medical professional, the General Manager may inform the family by telephone.

Exceptions for Hospice

If a resident receiving licensed hospice services has a valid written DNR and a written hospice plan that gives team members instructions on what to do if the resident stops breathing or his/her heart stops beating, the community team members may withhold CPR, and will not have to call "911". They must call the Hospice. If the hospice staff is not available to provide direction, then the community must immediately contact the resident's legal healthcare representative (DPOA) for direction. If no legal representative has been appointed or is not available, then the community must immediately call 911 to arrange for emergency transport and must initiate cardiopulmonary resuscitation.

Always call 911 unless the resident has a Hospice DNR Plan that indicates the Hospice should be called instead. Initiate CPR in the absence of a valid POLST/DNR form and continue CPR and first aid until paramedics or other qualified medical professionals (example: RN, LPN, MD, NP) arrive.

Physician Orders for Life-Sustaining Treatment (POLST)

FIRST follow these orders, **THEN** contact physician, nurse practitioner or PA-C. This is a Physician Order Sheet based on the person's medical condition and wishes. Any section not completed implies full treatment for that section. Everyone shall be treated with dignity and respect.

Last Name

First/Middle Initial

Date of Birth

A **CARDIOPULMONARY RESUSCITATION (CPR):** Person has no pulse and is not breathing.
☐ CPR/Attempt Resuscitation ☐ DNR/Do Not Attempt Resuscitation (Allow Natural Death)
 When not in cardiopulmonary arrest, follow orders in **B**, **C** and **D**.

B **MEDICAL INTERVENTIONS:** Person has pulse and/or is breathing.
☐ **COMFORT MEASURES ONLY** Use medication by any route, positioning, wound care and other measures to relieve pain and suffering. Use oxygen, oral suction and manual treatment of airway obstruction as needed for comfort. **Patient prefers no transfer: EMS contact medical control to determine if transport indicated.**
☐ **LIMITED ADDITIONAL INTERVENTIONS** Includes care described above. Use medical treatment, IV fluids and cardiac monitor as indicated. Do not use intubation, advanced airway interventions, or mechanical ventilation. **Transfer to hospital if indicated. Avoid intensive care if possible.**
☐ **FULL TREATMENT** Includes care described above. Use intubation, advanced airway interventions, mechanical ventilation, and cardioversion as indicated. **Transfer to hospital if indicated. Includes intensive care.**
 Additional Orders: (e.g. dialysis, etc.) _____

C **ANTIBIOTICS:**
☐ No antibiotics. Use other measures to relieve symptoms.
☐ Determine use or limitation of antibiotics when infection occurs, with comfort as goal.
☐ Use antibiotics if life can be prolonged.
 Additional Orders: _____

D **ARTIFICIALLY ADMINISTERED NUTRITION:** Always offer food and liquids by mouth if feasible.
☐ No artificial nutrition by tube.
☐ Trial period of artificial nutrition by tube. (Goal: _____)
☐ Long-term artificial nutrition by tube.
 Additional Orders: _____

E SUMMARY OF GOALS AND SIGNATURES

Discussed with:
☐ Patient ☐ Parent of Minor
☐ Health Care Representative
☐ Durable Power of Attorney for Health Care
☐ Court-Appointed Guardian
☐ Other: _____

Patient Goals/Medical Condition:

Print Physician/ARNP/PA-C Name

Phone Number

Patient/Resident or Legal Surrogate
for Health Care Signature
(mandatory)

Date

Physician/ARNP/PA-C Signature (mandatory)

Date

SEND FORM WITH PERSON WHENEVER TRANSFERRED OR DISCHARGED

Use of original form is strongly encouraged. Photocopies and FAXes of signed POLST forms are legal and valid

HIPAA PERMITS DISCLOSURE OF POLST TO OTHER HEALTH CARE PROVIDERS AS NECESSARY

Other Contact Information (Optional)

Name of Guardian, Surrogate or other Contact Person	Relationship	Phone Number	
Name of Health Care Professional Preparing Form	Preparer Title	Phone Number	Date Prepared

DIRECTIONS FOR HEALTH CARE PROFESSIONALS

Completing POLST

- Must be completed by a health care professional based on patient preferences and medical indications.
- POLST must be signed by a physician, nurse practitioner or PA-C to be valid. Verbal orders are acceptable with follow-up signature by physician or nurse practitioner in accordance with facility/community policy.
- Use of original form is strongly encouraged. Photocopies and FAXes of signed POLST forms are legal and valid.

Using POLST

- Any section of POLST not completed implies full treatment for that section.
- A semi-automatic external defibrillator (AED) should not be used on a person who has chosen "Do Not Attempt Resuscitation."
- Oral fluids and nutrition must always be offered if medically feasible.
- When comfort cannot be achieved in the current setting, the person, including someone with "comfort measures only," should be transferred to a setting able to provide comfort (e.g., pinning of a hip fracture).
- A person who chooses either "comfort measures only" or "limited additional interventions" should not be entered into a Level I trauma system.
- An IV medication to enhance comfort may be appropriate for a person who has chosen "Comfort Measures Only."
- Treatment of dehydration is a measure which may prolong life. A person who desires IV fluids should indicate "Limited Interventions" or "Full Treatment."
- A person with capacity or the surrogate (if patient lacks capacity) can revoke the POLST at any time and request alternative treatment.

Reviewing POLST

This POLST should be reviewed periodically and a new POLST completed if necessary when:

- (1) The person is transferred from one care setting or care level to another, or
- (2) There is a substantial change in the person's health status, or
- (3) The person's treatment preferences change.

To void this form, draw line through "Physician Orders" and write "VOID" in large letters.

Review of this POLST Form

Review Date	Reviewer	Location of Review	Review Outcome
			☐ No Change ☐ Form Voided ☐ New form completed
			☐ No Change ☐ Form Voided ☐ New form completed
			☐ No Change ☐ Form Voided ☐ New form completed

SEND FORM WITH PERSON WHENEVER TRANSFERRED OR DISCHARGED

Revised December 200

2.6 Prescribed Diets

.....Revised 05/01/2018 Reviewed 05/01/2018

Policy Purpose

Merrill Gardens offers residents options that allow for a limited range of special dietary needs. Residents must have the ability to follow their prescribed diet by making food selections that will support their diet restrictions. Merrill Gardens will assist with guiding residents receiving assisted living services to appropriate choices for diets we offer, but residents have the right to make choices that are not within the restrictions of their prescribed diet.

Refer to DS-Liberalized Diets.

Definitions

Liberalized diet

The relaxation of the diet prescription that may result in a better nutrition outcome for a patient and an improved quality of life. Beyond supporting improved quality of life where a resident enjoys food and beverages that are familiar and satisfying, a move toward liberalized diets may result in improved intake and a decrease in the malnutrition and unintended weight loss that often occurs when residents are served food they do not want. Following are the available liberalized diets:

REGULAR: Meets RDA for persons 51 and over. Includes a wide variety of foods to help ensure nutritional adequacy. 50-60% of calories from carbohydrates, 15-30% of calories from protein, and 30-35% calories of fat per day. Appropriate for most residents.

CONSISTENT CARBOHYDRATE (CCHO): Includes most of the foods allowed on the Regular Diet with routine use of sugar substitutes, diet syrups and diet jellies. No sugar packets served with meals. Allows some Regular desserts with meals, or for snacks/activities with consideration of consistent carbohydrate content. Is recommended for residents with a stable diabetic condition.

HEART HEALTHY: Based on the Regular Diet and intended for lowering the risk of developing heart disease by limiting the intake of fat, cholesterol and sodium. To prevent the diet from being too restrictive for the elderly, the fat is restricted to less than or equal to 30% of the total calories and the sodium is restricted to 2000-2500mg per day. Liberal use of salt-free seasonings is encouraged to produce a flavorful and appealing diet. If desired, salt substitutes must be physician ordered. Milk is limited to 2 cups per day and regular bread and margarine are limited to 3 times a day. Highly salted or cured foods are avoided. Limited amounts of salt are used in cooking.

Finger Foods

Foods that can be picked up with the fingers to be eaten. Not a liberalized diet but providing finger foods enables residents to maintain the freedom to eat independently for as long as possible. This can be particularly important for residents who may begin to lose their ability to properly use utensils.

Modified Diets

A mechanically altered diet is designed to permit easy chewing and swallowing. The general diet is modified in consistency and texture by cooking, grinding and chopping, mincing or mashing. The diet includes foods soft in texture such as cooked fruits and vegetables, mostly ground meats, soft breads and cereals and foods that easily dissolve in the mouth. It may also be recommended as a transitional diet from a liquid diet to a regular diet if recovering from surgery or a long illness. A mechanically altered diet may also be recommended for intestinal or stomach discomfort.

MECHANICAL SOFT: Cohesive, moist, semisolid foods that require some chewing ability. Included are fork-mashable fruits and vegetables. Meats are shredded, ground or chopped into less than ½ inch pieces. Excluded are most bread products, crackers, and other dry foods unless moistened with syrup, jelly, margarine, butter, milk, gravy or sauce. Cakes, cookies and pastries are soft and moist without dried fruits, coconut, nuts, pineapple or seeds.

PUREED: Pureed to the consistency of smooth, moist mashed potatoes or pudding like foods that require very little chewing ability.

Liquids (The cost of thickened liquids is to be borne by the resident):

NECTAR-THICK CONSISTENCY: easily pourable and is comparable to apricot nectar or thicker cream soups.

HONEY-THICK CONSISTENCY: slightly thicker, is less pourable, and can be drizzled from a cup or bowl.

AL: *The community will not provide thickened liquids. Residents who require thickened liquids will be reviewed for skilled nursing.*

Guidelines

Dietary Orders

- All meals will have a variety of options available so that a resident choosing to follow their own liberalized diet will have multiple food options available to them.
- Dining Service Staff will make menu choices for residents only if there is a written healthcare provider's diet order in that resident's chart. A copy of the healthcare provider's order will be filed in the Dining Services Department.
 - Diet Orders are only accepted when the prescribing healthcare provider directs that the diet cannot be liberalized to the point that the Resident can self-monitor.
 - The diet order shall be given to the Executive Chef or designee with a photo of the resident before the resident moves in, or as soon as practical when the order is received.
 - Diet Orders with the associated diagnosis will be reviewed by the RCD/GHD & Executive Chef at the community level.
 - The resident's diet with picture will be posted in a place where team members can review before service.

- The RCD/GHD will contact the prescribing healthcare provider to obtain information as to the intent of the diet. If a physician orders a specialized diet that is not on the Diet Order Form, the RCD/GHD or appropriate designee will be responsible to contact the physician to clarify the order.
- If a change cannot be made by the physician to a diet offered by the community, the physician and the resident will be notified that the community will not be able to meet the resident's specific dietary needs.
- The prescribed diet order is to be included on the Service Plan.
- The community's consulting dietitian will be contacted, as needed, for consultation in writing on the prescribed diet when this information is not provided by the healthcare provider. A fee may be associated with this consultation which, in most cases, will be the responsibility of the Resident.
- The GM may initiate a move-out notice if the resident's dietary needs cannot be met by the community.

Thickened Fluids

- All thickened liquids provided by the community will be ordered pre-thickened by the Executive Chef. Team Members will not thicken fluids for residents. The cost of these supplies will be the responsibility of the resident requiring the special consistency.
- Residents evaluated to be independent with their medications and capable of managing thickener may choose to thicken their fluids independently. These residents are not required to use the pre-thickened fluids provided by dining services. The cost for either will be the resident's responsibility.
- Residents with orders for thickened liquids will only be served pre-thickened fluids. If a resident refuses to comply with the physician's order, they will be responsible for pouring their own beverages. Team Members will not serve liquids in a form that does not comply with doctor's orders.

Resident Refusal of Prescribed Diet

- Residents may refuse liberalized diets as served and will be offered a reasonable substitute meal or meal replacement. The consequences of refusing the diet prescription will be explained to the resident by the RCD/GHD
- Refusal of the prescribed diet will be documented in the resident's progress notes and service plan if ongoing.
- A change in the diet order will be requested from the resident's healthcare provider.
- Texture-modified diets such as pureed, mechanical soft, and thickened liquids should not be liberalized unless a resident has had an improvement in chewing or swallowing ability. A Speech and Language Pathologist consult should be requested prior to changing texture modifications.
- Changes in prescribed diets must be written by the resident's healthcare provider before implementing.
- If the resident's healthcare provider refuses to change the diet order or if the resident continues to refuse the liberalized or modified diet, a care conference will be arranged to

involve the significant family member or guardian. This conference is to determine the best course of action going forward.

- For residents unwilling to comply with a prescribed diet, a Risk Agreement form is to be completed. Physician acknowledgement of this refusal will be kept in the resident's chart.



Diet Orders and Dietary Communication

Please fax back to:

Fax Number:

Resident Name:

Apartment Number:

- | | | | |
|---------------------------------------|--------------------------------------|--|---|
| ☐ New Resident | ☐ Expiration | ☐ To Hospital or HLOC | ☐ Out of Community |
| ☐ Diet Change | ☐ Weight Loss | ☐ Pressure Ulcer | ☐ Poor Intake |

Dear Dr.

Please read the diet principles listed below and indicate any diet orders for the above-named resident. In the absence of orders, the resident will self-monitor their dietary choices.

Merrill Gardens Diet Principles

Any Time Dining allows for the resident to visit the dining room from AM- PM permitting restaurant style dining. The resident is to self-manage their prescribed diet with appropriate choices being made available. Residents who need assistance with making appropriate choices will be offered guidance, but we are unable to restrict choices and access to foods and liquids not part of a prescribed diet. Garden Houses may have meal times and will have availability of snack foods between meals.

Specialized diets are limited to the following:

DIET ORDER:

☐ Regular

☐ Heart Healthy

☐ CCHO (Liberal Diabetic)

CONSISTENCY:

☐ Regular

☐ Mechanical Soft

☐ Puree (In Alabama, not permitted for swallowing issues)

THICKENED LIQUIDS (The cost of thickened liquids is to be borne by the resident):
(Not permitted in Alabama)

☐ Nectar-like Consistency

☐ Honey-like Consistency

☐ FINGER FOODS

☐ ADAPTIVE DEVICES for DINING:

FOOD ALLERGIES:

Date: _____ By: _____

Physician

Date: _____ By: _____

Community RCD or GHD

For Community Use:

Dining Location:

☐ Independent

☐ Assisted Dining

☐ Room Trays

☐ Dependent Dining

Available liberalized diets:

Regular: Meets RDA for persons 51 and over. Includes a wide variety of foods to ensure nutritional adequacy. 50-60% of calories from carbohydrates, 15-30% of calories from protein, and 30-35% calories of fat per day. Appropriate for most residents.

Consistent Carbohydrate (CCHO): Includes most of the foods allowed on the Regular Diet with routine use of sugar substitutes, diet syrups and diet jellies. No sugar packets served with meals. Allows some Regular desserts with meals, or for snacks/activities with consideration of consistent carbohydrate content. Is recommended for residents with a stable diabetic condition.

Heart Healthy: Based on the Regular Diet and intended for lowering the risk of developing heart disease by limiting the intake of fat, cholesterol and sodium. To prevent the diet from being too restrictive for the elderly, the fat is restricted to less than or equal to 30% of the total calories and the sodium is restricted to 2000-2500mg per day. Liberal use of salt-free seasonings is encouraged to produce a flavorful and appealing diet. If desired, salt substitutes must be physician ordered. Milk is limited to 2 cups per day and regular bread and margarine are limited to 3 times a day. Highly salted or cured foods are avoided. Limited amounts of salt are used in cooking.

Texture Modified Diets:

A mechanically altered diet is designed to permit easy chewing and swallowing. The general diet is modified in consistency and texture by cooking, grinding and chopping, mincing or mashing. The diet includes foods soft in texture such as cooked fruits and vegetables, mostly ground meats, soft breads and cereals and foods that easily dissolve in the mouth. It may also be recommended as a transitional diet from a liquid diet to a regular diet if recovering from surgery or a long illness. A mechanically altered diet may also be recommended for intestinal or stomach discomfort.

Mechanical Soft: Cohesive, moist, semisolid foods that require some chewing ability. Included are fork-mashable fruits and vegetables. Meats are shredded, ground or chopped into less than ½ inch pieces. Excluded are most bread products, crackers, and other dry foods unless moistened with syrup, jelly, margarine, butter, milk, gravy or sauce. Cakes, cookies and pastries are soft and moist without dried fruits, coconut, nuts, pineapple or seeds.

Pureed: Pureed to the consistency of smooth, moist mashed potatoes or pudding like foods that require very little chewing ability.

Liquids (The cost of thickened liquids is to be borne by the resident):

Nectar-thick consistency: easily pourable and is comparable to apricot nectar or thicker cream soups.

Honey-thick consistency: slightly thicker, is less pourable, and can be drizzled from a cup or bowl.



Prescribed Diet Release

I, _____ fully understand that

DR. _____

Has prescribed a diet for me that is part of a treatment regimen specifically related to my disease/diagnosis.

I have been informed of the risks/benefits that may occur as a result of noncompliance to the prescribed diet including, but not limited to decline in health and death.

As a part of my resident rights, I am requesting a diet of my own preference and release

Dr. _____ and _____ from
(name) (community)

any responsibility associated with this decision.

Signatures:

Resident or Responsible Party: _____

Relationship to Resident: _____

Community Representative: _____

Date: _____

